

R E V I E W

Conscious and updated mesotherapy: When informed consent becomes truly informed

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Abstract

Introduction: Informed consent is a central component of the patient–physician relationship, yet in clinical practice it is often reduced to a mere formal requirement. Mesotherapy, a minimally invasive medical treatment characterized by a high degree of personalization and clinical variability, represents a paradigmatic context in which to reconsider the true meaning of informed consent in contemporary medicine.

Objectives: This article aims to reframe informed consent in mesotherapy as a conscious, transparent, and genuinely informative process grounded in clinical dialogue and shared decision-making.

Methods: The key elements of the informed consent process in mesotherapy are discussed in light of recent scientific and regulatory developments in Italy. A practical checklist is proposed to guide the clinical interview.

Results: The analysis highlights the essential components of an effective informed consent process and provides a practical framework to support communication between physician and patient during mesotherapy consultations.

Conclusions: Mesotherapy emerges as an example of good clinical practice in which properly structured informed consent contributes to improving the quality and safety of care.

Key words: mesotherapy, informed consent, aesthetic medicine



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Introduction

Informed consent today represents one of the central elements of the therapeutic relationship; however, its current configuration is the result of a progressive historical, cultural, and legal evolution¹. In the past, medical practice was framed within a predominantly paternalistic model, in which therapeutic decisions were essentially entrusted to the physician, while the patient was assigned a marginal role. In this context, information served an ancillary function, and consent – when present – assumed a largely formal value, eventually being perceived as an inconvenient bureaucratic burden.

The progressive recognition of the individual's right to self-determination has profoundly transformed this framework^{2,3}. The physician–patient relationship has evolved into a model based on dialogue, mutual understanding, and shared clinical decision-making. Within this new paradigm, informed consent is no longer conceived as a mere authorization for a medical act, but rather as the structured informational record of the clinical dialogue, aimed at enabling the patient to actively participate in and become fully aware of therapeutic choices. This process culminates in the signing of the shared information by both the physician and the patient, as active participants in a bilateral informational pathway.

In recent years, this approach has found solid confirmation at the regulatory level – most notably with Law No. 219/2017 – as well as within professional ethical codes, consolidating the notion that informed consent represents the point of convergence between two fundamental rights of the individual: the right to health and the right to self-determination^{4,5}.

At the same time, informed consent has increasingly been understood as a dynamic process that accompanies the entire course of care and must be adapted to the patient's clinical condition, level of understanding, and the evolving availability of therapeutic options. Information, therefore, cannot be conceived as a single, fixed act, but rather as an ongoing dialogue, open to updates and integrations over time. Moreover, the growing availability of health-related information – often disseminated through digital media and social networks – exposes patients to heterogeneous content,

within which the physician is called upon to act as a guarantor of quality, appropriateness, and comprehensibility of information.

Within this perspective, the principle has progressively emerged that the time devoted to communication between physician and patient constitutes time of care. This time requires attentive listening, clarity of explanation, and the ability to tailor information to the individual clinical case. It represents a qualifying moment of the medical act, with an orienting function that helps to realign patient expectations within the limits of scientific evidence and responsible clinical practice. As such, it serves as a quality indicator of the therapeutic relationship, capable of influencing both the appropriateness of therapeutic decisions and the protection of both patient and physician.

It therefore appears that this issue assumes particular relevance⁶ in treatments such as mesotherapy, which are characterized by intrinsic complexity⁷, variability of indications and outcomes, and a concrete risk of trivialization in common perception. This article should be interpreted as a conceptual and narrative analysis of informed consent in mesotherapy. The discussion is based on the review of relevant scientific literature, current Italian regulatory frameworks, and the most recent international consensus documents on mesotherapy practice. On this basis, a practical checklist was developed by the authors to support clinicians during the informed consent process. The checklist is not intended to represent a rigid protocol, but rather a structured guide derived from the integration of clinical experience, medico-legal principles, and current professional recommendations.

The current context of mesotherapy: between social media, trivialization of treatment, and emerging risks

In recent years, mesotherapy has experienced increasing diffusion, also due to its well-known characteristics of minimal invasiveness and ease of repeatability. This expansion, already highlighted in previous critical analyses of mesotherapy practice, has contributed to a progressive distortion of the perception of the treatment, which is increasingly regarded as a

standardized approach devoid of real clinical complexity⁸. A decisive role in this process is played by health-related communication conveyed through social media and marketing strategies⁹.

In these contexts, the narrative surrounding mesotherapy often emphasizes the immediacy of results and the simplicity of execution, while marginalizing response variability, limitations of efficacy, and potential risks. This type of communication fosters the development of unrealistic expectations in patients, who may approach the clinical consultation with preconceived beliefs that are not always consistent with accurate medical and scientific information^{10,11}, thereby complicating the shared decision-making process.

At the same time, the growing demand for minimally invasive treatments has facilitated phenomena of professional boundary transgression and abuse of medical practice^{12,13}. In some settings, mesotherapy is proposed or performed outside an appropriate medical framework, with an underestimation of the clinical, pharmacological, and medico-legal implications associated with injection-based procedures. This scenario further contributes to the trivialization of the technique and reinforces the erroneous perception of mesotherapy as a simple and risk-free procedure^{14,15}.

The very definition of mesotherapy as a “minimally invasive” treatment presents an inherent ambivalence. While it correctly describes the absence of major surgical access, it may simultaneously lead to an underestimation of the medical nature of the procedure, which nonetheless involves disruption of the skin barrier, the administration of biologically active substances, and the possibility of adverse events, including clinically significant ones. This minimization of perceived risk often translates into reduced attention to clinical assessment and patient information¹⁴.

Within this framework, recent alerts issued by regulatory authorities in Europe have highlighted concrete concerns related to the improper use of products in aesthetic and mesotherapeutic settings. In particular, the Spanish Agency for Medicines and Medical Devices has reported cases of injectable administration of substances marketed as cosmetics, underscoring the risk of confusion between cosmetic products and medicinal products, with serious implications for patient

safety¹⁶. These reports confirm that mesotherapy cannot be considered a practice devoid of regulatory relevance, but rather one that requires medical expertise, prescribing appropriateness, and rigorous patient information¹⁷.

In this scenario, informed consent assumes a decisive role not only as a legal obligation, but also as an essential clinical tool to reposition treatment within a framework of awareness, responsibility, and mutual protection between physician and patient, in line with the most recent standards of safe and evidence-based practice^{18,19}.

What is mesotherapy today? updated definition and clinical scope according to the 2025 consensus

Mesotherapy has long been described as a technique involving intradermal microinjections of active substances, with definitions primarily focused on procedural aspects, progressively losing clarity regarding the true clinical and professional framework of the practice. More recent scientific and regulatory developments have made it necessary to move beyond these overly generic definitions. According to the *International Consensus Guidelines on the Safe and Evidence-Based Practice of Mesotherapy*¹⁸, mesotherapy should now be defined as a medical act consisting of the administration, via intradermal or superficial microinjections, of authorized medicinal products or medical devices, performed for preventive, therapeutic, or rehabilitative purposes, based on an individual clinical assessment and within a structured, evidence-based medical pathway¹⁸.

This definition represents a significant conceptual shift, moving the focus from the technique itself to the clinical context and professional responsibility that characterize the mesotherapeutic act. The injection component remains the defining feature of mesotherapy²⁰, involving disruption of the skin barrier and administration of biologically active substances. Mesotherapy should not be equated with topical procedures or advanced cosmetic practices, but must be fully framed within the scope of healthcare services reserved for physicians.

The biological rationale of mesotherapy is based on the local modulation of the tissue microenvironment, achieved through the combination of the route of administration, the concentration of the substance (active or medical device), and the individual biological response. Under certain clinical conditions, intradermal or superficial injection allows for targeted action, with a potential systemic-sparing effect. However, treatment efficacy is neither uniform nor absolutely predictable and depends on multiple factors, including the underlying condition, patient characteristics, the type of substance used, and the adopted protocol^{18,20}.

Clinical indications for mesotherapy must therefore be formulated with caution and always grounded in a case-specific evaluation. The method is neither universally effective nor confined to rigidly standardized procedures. Its limitations, as well as the possibility of partial or absent response, are intrinsic elements of the treatment and must be considered an integral part of the decision-making process. This aspect is particularly relevant in contexts in which mesotherapy is proposed as a simple or rapid solution to complex conditions, with the risk of generating unrealistic patient expectations^{9,10}.

Accordingly, the mesotherapeutic act involves the use of authorized medicinal products or medical devices and the performance of microinjections, with potential local and systemic risks, requiring clinical, pharmacological, and management competencies that fall exclusively within the scope of medical responsibility, with inevitable medico-legal implications²¹. Recognition of the method as a medical act thus highlights an indispensable sequence: a correct clinical indication, an adequate preliminary assessment, and complete, personalized patient information.

Why mesotherapy requires a specific informed consent

For these reasons, mesotherapy requires a well-structured informed consent that cannot be reduced to generic models applicable to any healthcare intervention. Unlike more standardized procedures, mesotherapy is based on personalized therapeutic choices involving the clinical indication, the authorized

medicinal products or medical devices to be used, the routes of administration, the depth and frequency of microinjections, and the overall duration of treatment. These choices arise from an individual clinical assessment and render the treatment intrinsically variable, both in its design and in the expected outcomes¹⁸.

This complexity is further compounded by the not infrequent use of medicinal products administered through routes not explicitly indicated in the summary of product characteristics (off-label use), by the combination of multiple products, and by the use of authorized medical devices within protocols tailored to the individual clinical case. In such circumstances, while remaining within the physician's professional autonomy and responsibility, the duty to inform is further reinforced and must include an explanation of the clinical rationale for the chosen approach, the available therapeutic alternatives, the limits of the scientific evidence, and any uncertainties related to individual patient response. Specific informed consent thus becomes the instrument through which these decisions are made comprehensible and documented, in accordance with the regulatory principles governing off-label use and non-rigidly standardized practices²².

Despite its minimally invasive nature, mesotherapy entails predictable local reactions and rare but possible adverse events. The minimally invasive nature of the procedure does not exclude the possibility of clinically relevant complications, nor does it diminish the obligation to provide complete and proportionate information. The communication of risks - both general and specific - even when statistically infrequent, represents an essential component of informed consent and a necessary condition for the patient's decision to be truly informed.

To appropriately contextualize patient expectations, due attention must be paid to the influence of simplified and promotional communication channels, which may lead patients to anticipate rapid, certain, or side-effect-free outcomes. The informational interview preceding the signing of the consent form thus allows these expectations to be realigned with the actual potential of the technique, clarifying the need for treatment cycles and the possibility of outcomes that may not correspond to initial expectations. In this sense, informed consent does not merely legitimize the

medical act, but actively contributes to the quality of the therapeutic relationship and to the sustainability of the treatment pathway.

In light of these considerations, informed consent in mesotherapy should be understood as a true clinical, ethical, and legal safeguard. It protects the patient's right to self-determination by ensuring a free and informed choice based on personalized information, while simultaneously protecting the physician by documenting the appropriateness of the informational and decision-making process. In a treatment characterized by complexity²³, variability, and areas of uncertainty, specific informed consent represents the point of balance between personalization of care and professional responsibility, and constitutes an indispensable element of good clinical practice in mesotherapy¹⁸.

From formal informed consent to conscious, transparent, and informative consent

It may therefore be considered appropriate that generic and indistinct formulations give way to a conscious, transparent, and genuinely informative process, proportionate to the specificity of the treatment and the preceding decision-making pathway. This is not about introducing new categories of consent but about qualifying the manner in which informed consent is constructed and documented, so that it aligns with the clinical, relational, and decisional complexity of mesotherapy.

A key element of this process is the centrality of the information provided by the patient. The medical history does not constitute a merely formal preliminary step but is an integral part of the informational and decisional pathway that leads to the indication or contraindication of the treatment. Previous radiotherapy or chemotherapy, treatments with immunosuppressive drugs, known or suspected allergies, autoimmune or genetic diseases, hypersensitivity conditions, ongoing or recent pharmacological therapies, as well as any adverse reactions to previous injectable treatments, directly influence the choice of products, the methods of performing mesotherapy, or the decision not to proceed with the treatment.

Alongside strictly clinical elements, the patient's cosmetological habits and self-care routines also assume relevance. The habitual use of cosmetics, aesthetic treatments, at-home or professional procedures, as well as the degree of consistency and attention devoted to these practices, provides useful indications regarding the individual approach to managing one's health and appearance. Collectively, this information helps guide treatment planning, the selection of follow-up modalities, and the evaluation of adherence to post-treatment instructions.

A truly conscious and transparent informed consent requires a realistic representation of the benefits and limitations of the treatment²⁴. Expected outcomes, the absence of guaranteed results, the possibility of partial or absent responses, and the need for repeated cycles must be clarified. Within this framework, the economic²⁵ and temporal aspects of the treatment cannot be overlooked. The total cost of the mesotherapy program, its duration over time, the commitment required in terms of sessions and follow-ups, as well as compatibility with the patient's financial and time availability, are all relevant for a truly informed decision. Omitting these elements risks compromising treatment adherence and generating interruptions or dissatisfaction unrelated to clinical efficacy but rather to an incomplete initial assessment.

Equally essential is the ability to document the information collected, the shared evaluations, and the choices made, thereby restoring to consent its authentic meaning as a documented outcome of a bilateral informational pathway. In the past, the moment of signing sometimes appeared as an awkward formality, one that could disrupt the trust that had been built, rather than express a genuine choice. Today, the final signature does not exhaust consent but marks its concluding moment, certifying that the decision was reached within a structured and responsible clinical dialogue.

During each session, the physician should verify the stability of the clinical conditions in order to allow the patient to confirm the consent already given.

From this perspective, mesotherapy particularly illustrates how consent fully qualifies as an instrument capable of influencing the quality of the care relationship and, consequently, the level of care itself.

Proposed checklist for informed consent in mesotherapy

The structure of the proposed informed consent checklist is consistent with the principles expressed in the current recommendations of the Italian Society of Mesotherapy (SIM), which recognize the central role of structured patient information in the safe and responsible practice of mesotherapy. If consent does not merely precede or follow the medical act, but accompanies the entire decision-making process through a structured

dialogue, guiding its content and making explicit the method by which information is shared^{26,27}, the final signature no longer represents a separate or merely formal step, but the natural outcome of a process already completed, in which physician and patient have followed the same informational pathway together.

From this perspective, the process is recognized as relational, in which information has been genuinely understood, discussed, and progressively constructed. Table 1 presents a practical checklist to guide clinicians through the informed consent process in mesotherapy,

Table 1. This table is a practical tool to support clinicians in making the informed consent process complete, traceable, and proportionate to the complexity of the procedure — without replacing clinical judgment or imposing a rigid protocol.

Consent Area	Content to Customize in Consultation and Form	Clinical and Legal Purpose
1. Clinical Framing	Pathology or condition to be treated. Clinical rationale for mesotherapy in the individual case.	Medical indication/clinical diagnosis.
2. Relevant Medical History	Previous radiotherapy or chemotherapy, oncological diseases, known or suspected allergies, autoimmune or genetic diseases, ongoing or recent pharmacological therapies, previous adverse reactions to injectable treatments.	Demonstrates that the therapeutic choice also derives from patient-provided information and that medical history is an integral part of consent.
3. Nature of the Mesotherapy Act	Description of mesotherapy as a medical procedure involving intradermal or superficial microinjections. Clarification of its minimally invasive but medical character.	To prevent trivialization of the procedure and clarify the medical nature of the act.
4. Products Used	Specification of authorized drugs or medical devices employed. Any off-label use and clinical rationale. Exclusion of cosmetic products for injectable use.	Regulatory transparency and basis for specific consent.
5. Clinical Traceability	Documentation in the medical record of products, dosages, injection sites, sessions performed, and outcomes.	Continuity of care and professional protection.
6. Execution Modalities	Approximate number of sessions, frequency, injection sites, possibility of protocol adjustments based on clinical response.	To make the patient aware of the treatment structure and its potential evolution.
7. Expected Benefits	Realistic clinical improvements. No guarantee of results. Possibility of partial or absent response.	To prevent unrealistic expectations and disputes related to outcomes.
8. Therapeutic Alternatives	Other available options, including the possibility of foregoing treatment.	To reinforce self-determination and freedom of choice.
9. General and Specific Risks and Adverse Events	Predictable risks (local reactions, pain, bruising). Rare or unpredictable risks. Methods for managing complications.	Ensure consent remains valid even in the absence of biological damage.
10. Post-Treatment Instructions	Recommended behaviors, warning signs, and contact methods in case of adverse events.	Continuity of care and patient safety.
11. Patient Expectations	Verification and discussion of subjective expectations regarding the treatment.	Align patient perception with clinical reality.
12. Consent/Refusal and Revocability	Signature as the outcome of a bilateral informational process. Explicit possibility of revocation at any time.	Proper closure of the informational process.

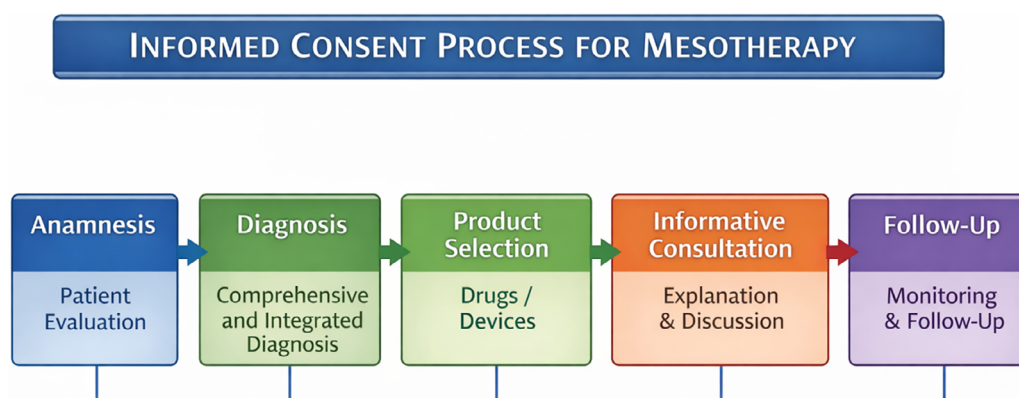


Figure 1. Schematic representation of the informed consent process in mesotherapy. The figure illustrates a structured and sequential consultation pathway, starting from comprehensive and integrated diagnosis, followed by product selection, informative consultation, and iterative clinical decision-making. The process integrates clinical assessment, patient-provided data, risk-benefit evaluation, and shared discussion, ultimately leading to informed consent signature and structured follow-up.

highlighting areas for customization and their clinical-legal purpose. The final signature certifies that the patient has actively participated in the decision-making process and that the physician has fulfilled their informational duty in a personalized and proportionate manner (Figure 1). Consent thus becomes the written testimony of an authentic clinical dialogue, in which therapeutic choices were not imposed but shared, and in which the patient's self-determination is expressed within a conscious and responsible care relationship.

Conclusions

The process developed in this work demonstrates that the informational pathway of consent represents a central node in the care process. It is at this intersection that professional competence, ethical responsibility, and the right to personal self-determination converge. When consent is reduced to a mere signature, it loses its function; when it is constructed as a process, it gains real clinical value. Mesotherapy, with its characteristics as a minimally invasive yet complex treatment, particularly highlights this necessity.

The variability of indications, the personalization of treatment protocols, the use of authorized drugs or medical devices, the possibility of off-label applications, and the absolute unpredictability of outcomes all

demand a model of consent that is genuinely informative, conscious, and transparent. In this sense, mesotherapy serves as a paradigmatic example, illustrating the critical issues and the potential of informed consent in contemporary medicine.

Informed consent finds its full expression when it is based on a structured consultation. Here, clinical information is intertwined with patient-provided data. Medical history, lifestyle habits, expectations, financial resources, and available time are not marginal elements but variables that concretely guide therapeutic decisions. Consent thus becomes the space where medical knowledge meets the uniqueness of the individual.

From this perspective, the checklist proposed in this article acts as a methodological and customizable guide for the clinical consultation. It makes the informational pathway explicit, promotes completeness and traceability of shared information, and restores to the final signature its authentic meaning: not as a defensive act, but as the natural conclusion of a process already understood and internalized.

Conceived in this way, informed consent does not burden clinical practice but improves its quality. It strengthens the care relationship and reduces the risk of misunderstandings and conflicts. Ultimately, speaking of "truly informed consent" means reaffirming a vision of medicine based on responsibility, transparency, and respect for the individual. When practiced

within this framework, mesotherapy becomes not only a technically appropriate treatment but also a mature exercise of relational medicine.

It is through this process which is grounded in dialogue, transparency, and shared understanding, that informed consent regains its most authentic function: not merely to authorize an intervention, but to affirm the quality of the care relationship itself.

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